BIOCITY HEALTHCARE
Preventive Health Care Company

Smart Report

Visit ID	**: MITD63340**	Registration	: 26/Apr/2024 03:17PM
UHID/MR No	: AITD.0000063250	Collected	: 26/Apr/2024 03:17PM
Patient Name	**: Mr.GURPREET SINGH**	Received	: 26/Apr/2024 06:37PM
Age/Gender	: 26 Y 0 M 0 D /M	Reported	: 27/Apr/2024 10:25AM
Ref Doctor	: Dr.SELF	Status	: Final Report
Client Name	: NIVA BUPA	Client Code	: 144
Client Add	: INDIA	Barcode No	: 10150866

DEPARTMENT OF HAEMATOLOGY
AHC PACKAGE 3

CBC+ ESR

Sample Type : WHOLE BLOOD EDTA

Test Name	Result	Unit	Bio. Ref. Range	Method
HAEMOGLOBIN (HB)	15.0	gm/dl	13.00-17.00	Non-Cyanmethemoglobin
TOTAL LEUCOCYTE COUNT (TLC)	7,910	cell/cmm	4000-10000	Flow cytometry
DLC (by Flow cytometry/Microscopy)				
NEUTROPHIL	50.8	%	40-75	
LYMPHOCYTE	36.8	%	20-40	
EOSINOPHIL	2.7	%	01-07	
MONOCYTE	9.4	%	2-10	
BASOPHIL	0.3	%	00-02	
ABSOLUTE NEUTROPHIL COUNT	4.02	x10^3 Cells/uL	1.5-7.8	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	2.91	x10^3 Cells/uL	2.0-3.9	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.21	x10^3 Cells/uL	0.2-0.5	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.74	x10^3 Cells/uL	0.2-0.95	Automated Calculated
ABSOLUTE BASOPHIL COUNT	0.02	x10^3 Cells/uL	0.02-0.2	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	4.86	million/cmm	4.50-5.50	Optical Flowcytometry
PCV/HAEMATOCRIT	45.3	%	40-50	RBC pulse height detection
MCV	93.2	fL	80-100	Automated/Calculated
MCH	30.8	pg	27-32	Automated/Calculated
MCHC	33.0	g/dl	32-36	Automated/Calculated

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DEPARTMENT OF HAEMATOLOGY
AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method

Hemoglobin (Hb)

Hemoglobin, found within red blood cells, functions to transport oxygen and lends the characteristic red color to blood cells. The hemoglobin test quantifies the level of hemoglobin in the blood, serving as an effective gauge of the blood's oxygen-carrying capacity throughout the body.

Conditions associated with elevated ESR include acute inflammatory processes, acute and chronic infections, tissue damage (necrosis), rheumatoid arthritis, collagen diseases, malignancies, stress, pregnancy, and more.

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DEPARTMENT OF HAEMATOLOGY
AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
HBA1C				
Sample Type : WHOLE BLOOD EDTA				
HBA1c	5.2	%	Normal Glucose tolerance ~(non diabetic):<5.6% Pre-diabetic: 5.7-6.4% Diabetic Mellitus: >6.5%	HPLC
ESTIMATED AVG. GLUCOSE	102.54	mg/dl	62.30 - 142.72	

INCREASED IN

1. Chronic renal failure with or without hemodialysis.
2. Iron deficiency anemia. Increased serum triglycerides.
3. Alcohol.
4. Salicylate treatment.

DECREASED IN

1. Shortened RBC life span (hemolytic anemia, blood loss), Pregnancy.
2. Ingestion of large amounts (>1g/day) of vitamin C or E.
3. Hemoglobinopathies (e.g.: spherocytes) produce variable increase or decrease.
4. Results of %HbA1c are not reliable in patients with chronic blood loss and consequent variable erythrocyte life span.

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DEPARTMENT OF BIOCHEMISTRY
AHC PACKAGE 3

LIVER FUNCTION TEST

Sample Type : SERUM

Test Name	Result	Unit	Bio. Ref. Range	Method
TOTAL BILIRUBIN	0.48	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	0.13	mg/dl	0.00-0.30	Diazotized, Sulfanilic
UNCONJUGATED (I.D. Bilirubin)	0.35	mg/dl	0.1-1.0	Calculated
SGOT	**42**	U/L	0.0-31.0	Enzymatic,IFCC
S.G.P.T	**75**	U/L	10.0-35.0	Enzymatic,IFFC
ALKALINE PHOSPHATASE	95	U/L	30-120	IFCC MODIFIED METHOD
TOTAL PROTEINS	7.96	gm/dl	5.70-8.0	Biuret
ALBUMIN	4.35	gm/dl	3.50-5.30	BCG
GLOBULIN	3.61	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.2		1.0-2.0	Calculated
GGT	**76**	U/L	8.0-55	SZASZ IFCC

Comments and Interpretation:
The liver filters and processes blood as it circulates through the body. It metabolizes nutrients, detoxifies harmful substances, makes blood clotting proteins, and performs many other vital functions. The cells in the liver contain proteins called enzymes that drive these chemical reactions. When liver cells are damaged or destroyed, the enzymes in the cells leak out into the blood, where they can be measured by blood tests Liver tests check the blood for two main liver enzymes.

- **Aspartate aminotransferase (AST),SGOT:** The AST enzyme is also found in muscles and many other tissues besides the liver.
- **Alanine aminotransferase (ALT), SGPT:** ALT is almost exclusively found in the liver. If ALT and AST are found together in elevated amounts in the blood, liver damage is most likely present.

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DEPARTMENT OF BIOCHEMISTRY

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method

- **Low albumin levels may be caused by:** 1.A poor diet (malnutrition). 2.Kidney disease. 3.Liver disease.
- **High albumin levels may be caused by:** Severe dehydration.

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DEPARTMENT OF BIOCHEMISTRY

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
CALCIUM TOTAL				
Sample Type : Serum				
SERUM TOTAL CALCIUM	9.52	mg/dl	8.3-10.3	Arsenazo III

INTERPRETATION:

-Calcium level is increased in patients with hyperparathyroidism, Vitamin D intoxication, metastatic bone tumor, milk-alkali syndrome, multiple myeloma, Paget's disease.
-Calcium level is decreased in patients with hemodialysis, hypoparathyroidism (primary, secondary), vitamin D deficiency, acute pancreatitis, diabetic Keto-acidosis, sepsis, acute myocardial infarction (AMI), malabsorption, osteomalacia, renal failure, rickets.

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AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
LIPID PROFILE				
Sample Type : SERUM				
TOTAL CHOLESTEROL	192.6	mg/dl	Desirable : 80-200~Borderline: 200 – 239~High : >=240	Cholesterol oxidase/peroxidase
TRIGLYCERIDES	106.76	mg/dl	40-149~BorderLine : 150-199~High : 200-499~Very High : >=500	Glycerol phosphate oxidase/peroxidase
H D L CHOLESTEROL	48.31	mg/dl	40-60	Phosphotungstate/Mg-Cholesterol oxidase/ peroxidase
VLDL	21.35	mg/dl	15-30	Calculated
L D L CHOLESTEROL	**122.94**	mg/dl	70-106~Above Optimal : 100-129~Borderline High : 130-159~High : 160-189~Very High : >=190	Calculated
NON HDL CHOLESTEROL	**144.29**	mg/dl	Desirable: <130~BorderLine : 150-199~High : 200-499~Very High : >=500	Calculated
LDL / HDL RATIO	2.54			Calculated
T. CHOLESTEROL/ HDL RATIO	3.99			Calculated

Comment and interpretation:
A Lipid test can help determine your risk of the buildup of fatty deposits (plaques) in your arteries that can lead to narrowed or blocked arteries throughout your body (atherosclerosis).High cholesterol usually causes no signs or symptoms. A complete cholesterol test is done to determine whether your cholesterol is high and to estimate your risk of heart attacks and other forms of heart disease and diseases of the blood vessels.
A complete Lipid test includes the calculation of four types of fats in your blood:

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DEPARTMENT OF BIOCHEMISTRY

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
KFT WITH ELECTROLYTE				
Sample Type : SERUM				
SERUM UREA	**44.28**	mg/dL	16-43	Urease GLDH
Blood Urea Nitrogen (BUN)	20.69	mg/dl	5-25	
SERUM CREATININE	0.73	mg/dl	0.80–1.30	Sarcosine Oxidase
SERUM URIC ACID	5.7	mg/dl	3.6-8.20	Uricase Peroxidase
BUN/CREATININE RATIO	**28.34**		10-20	
Estimated Glomerular Filtration Rate (eGFR)	110.00	mL/min/1.73m2	REFER INTERPRETAION	
Electrolyte Profile				
SERUM CHLORIDE	105.3	mmol/L	98.0-107	ISE
SERUM SODIUM	142.0	mmol/L	135.0-145.0	ISE
SERUM POTASSIUM	4.52	mmol/L	3.50-5.10	ISE

Comment and Interpretation:

Healthy kidneys remove wastes and excess fluid from the blood. Blood and urine tests show how well the kidneys are doing their job and how quickly body wastes are being removed. Urine tests can also detect whether the kidneys are leaking abnormal amounts of protein, a sign of kidney damage.

- **Blood Urea / Urea nitrogen** comes from the breakdown of protein in the foods you eat. As kidney function decreases, the BUN level rises
- **Serum Creatinine** is a waste product that comes from the normal wear and tear on muscles of the body. Creatinine levels in the blood can vary depending on age and body size. A creatinine level of greater than 1.2 for women and greater than 1.4 for men may be an early sign that the kidneys are not working properly. As kidney disease progresses, the level of creatinine in the blood rises

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DEPARTMENT OF BIOCHEMISTRY

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
FBS				
Sample Type : Plane Blood				
Blood Sugar	85.52	mg/dl	70 - 110	Glucose Oxidase/Peroxidase

INTERPRETATION:

Increased In

- Diabetes Mellitus
- Stress (e.g., emotion, burns, shock, anesthesia)
- Acute pancreatitis
- Chronic pancreatitis
- Wernicke encephalopathy (vitamin B1 deficiency)
- Effect of drugs (e.g. corticosteroids, estrogens, alcohol, phenytoin, thiazides)

Decreased In

- Pancreatic disorders
- Extrapancreatic tumors
- Endocrine disorders
- Malnutrition
- Hypothalamic lesions
- Alcoholism
- Endocrine disorders

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DEPARTMENT OF HORMONE ASSAYS

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
TSH - THYROID STIMULATING HORMONE				
Sample Type : Serum				
TSH	3.46	ulU/mL	0.35-5.50	CLIA

INTERPRETATION:

1. Serum T3, T4 and TSH are the measurements form three components of thyroid screening panel and are useful in diagnosing various disorders of thyroid gland function.
2. Primary hyperthyroidism is accompanied by elevated serum T3 and T4 values along with depressed TSH levels.
3. Primary hypothyroidism is accompanied by depressed serum T3 and T4 values and elevated serum TSH levels.
4. Normal T4 levels accompanied by high T3 levels are seen in patients with T3 thyrotoxicosis. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness, malnutrition, renal failure and during therapy with drugs like propanolol and propylthiouracil.
5. Although elevated TSH levels are nearly always indicative of primary hypothyroidism, rarely they can result from TSH secreting pituitary tumors (secondary hyperthyroidism).
6. Low levels of Thyroid hormones (T3, T4 & FT3, FT4) are seen in cases of primary, secondary and tertiary hypothyroidism and sometimes in non-thyroidal illness also.
7. Increased levels are found in Grave's disease, hyperthyroidism and thyroid hormone resistance.
8. TSH levels are raised in primary hypothyroidism and are low in hyperthyroidism and secondary hypothyroidism.
9. **REFERENCE RANGE**

PREGNANCY	TSH in uIU/mL
1st Trimester	0.60 - 3.40
2nd Trimester	0.37 - 3.60
3rd Trimester	0.38 – 4.04

Age	TSH in uIU/mL
0 – 4 Days	1.00 - 39.00
2 Weeks to 5 Months	1.70 – 9.10
6 Months to 20 Yrs.	0.70 – 6.40
>55 Yrs.	0.50 - 8.90

(References range recommended by the American Thyroid Association)

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Ref Doctor	: Dr.SELF	Status	: Final Report
Client Name	: NIVA BUPA	Client Code	: 144
Client Add	: INDIA	Barcode No	: 10150866

DEPARTMENT OF HORMONE ASSAYS

AHC PACKAGE 3

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN D25 HYDROXY				
Sample Type : SERUM				
VITAMIN D	**22.96**	ng/ml	30-100	CLIA

INTERPRETATION:

LEVEL	REFERENCE RANGE
Deficiency (serious deficient)	< 10 ng/ml
Insufficiency (Deficient)	10-30 ng/ml
Sufficient (adequate)	30-100 ng/ml
Toxicity	> 100 ng/ml

DECREASED LEVELS:
-Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany.
-Inadequate exposure to sunlight.
-Dietary deficiency.
-Vitamin D malabsorption.
-Severe Hepatocellular disease.
-Drugs like Anticonvulsants.
-Nephrotic syndrome.
INCREASED LEVELS:
-Vitamin D intoxication.
COMMENTS:
-Vitamin D (Cholecalciferol) promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1, 25 Dihydronxy vitamin D (5-8 hrs).
-The assay measures D3 (Cholecaciferol) metabolites of vitamin D.
-25 (OH) D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
-Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.

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Preventive Health Care Company

Smart Report

Visit ID	**: MITD63340**	Registration	: 26/Apr/2024 03:17PM
UHID/MR No	: AITD.0000063250	Collected	: 26/Apr/2024 03:17PM
Patient Name	**: Mr.GURPREET SINGH**	Received	: 26/Apr/2024 06:37PM
Age/Gender	: 26 Y 0 M 0 D /M	Reported	: 27/Apr/2024 10:42AM
Ref Doctor	: Dr.SELF	Status	: Final Report
Client Name	: NIVA BUPA	Client Code	: 144
Client Add	: INDIA	Barcode No	: 10150866

DEPARTMENT OF CLINICAL PATHOLOGY

AHC PACKAGE 3

URINE EXAMINATION COMPLETE

Sample Type : URINE

Test Name	Result	Unit	Bio. Ref. Range	Method
PHYSICAL EXAMINATION				
QUANTITY	20	ml	0-50	
COLOUR	PALE YELLOW		PALE YELLOW	
TRANSPARENCY	CLEAR		Clear	
SPECIFIC GRAVITY	1.025		1.001 - 1.030	
CHEMICAL EXAMINATION				
pH	6.0		5-7	Double Indicator
PROTEIN	NIL		Nil	Protein - error of Indicators
REDUCING SUGAR	NIL		Nil	GOD-POD
UROBILINOGEN	NIL		Nil	Ehrlichs Reaction
KETONE BODIES	NIL		Nil	Legals Nitroprasside
BILIRUBIN	NIL		Nil	Azo-coupling Reaction
BLOOD	NIL		Nil	Pseudo-peroxidase
LEUCOCYTE	NIL		Nil	by an azo-coupling reaction
NITRITE	NIL		Nil	Diazotization Reaction
MICROSCOPIC EXAMINATION				
PUS CELLS	2-4	cells/HPF	0-5	
RBCs	NIL	Cells/HPF	Nil	

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